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IMPACT OF SEXUAL ORIENTATION AND GENDER IDENTITY ON UNWANTED SEXUAL EXPERIENCES AND DISCLOSURE DECISIONS

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Abstract – This project aims to further understand the barriers and experiences of female LGBTQIA+ survivors of sexual assault in disclosing their experience. Studies have shown that queer women experience sexual assault at higher rates, face more victim blaming, and report more negative mental health outcomes following an assault than straight women (Canan et al., 2021; Ford & Soto-Marquez, 2016; Long et al., 2007; Martin et al., 2011). To gain a deeper understanding of these trends, three focus groups of three participants were conducted. Groups were separated by sexual assault history and disclosure: two groups of LGBTQIA+ survivors and one group of LGBTQIA+ individuals who have not experienced sexual assault. This study will provide insight on the experiences of this at-risk group. The information gained from this study will allow the creation of better interventions and resources for LGBTQIA+ survivors of sexual assault.

Keywords: sexual assault, sexual orientation, gender identity, disclosure

In the United States, 11-17% of women and 2-3% of men report experiencing sexual assault (Basile et al., 2007; Tjaden & Thoennes, 2000).(Basile et al., 2007; Tjaden & Thoennes, 2000). Experiences with sexual assault lead to a variety of negative mental health outcomes including depression, anxiety, substance use, and suicidality (Carey et al., 2018; Chang et al., 2015; Coulter et al., 2017; Sabina & Ho, 2014). Resources to help survivors work through their experience and manage mental health struggles exist; however, to access these resources a survivor must seek help and disclose their experience. In some cases, disclosure can lead to additional victimization through negative experiences with law enforcement or support services (Campbell, 2008),(Campbell, 2008), but generally formal and informal disclosure of a sexual assault experience are helpful in the coping process (Ullman & Peter-Hagene, 2014).(Ullman & Peter-Hagene, 2014). Lack of formal disclosure may hinder survivor coping and recovery, and due to the negative mental health outcomes associated with experiencing sexual assault, it is important to understand more about why this process may not occur generally and in specific at-risk populations.

The entire experience of sexual assault, ranging from prevalence to reactions to help-seeking, is affected by one's identity. In particular, the literature suggests that gender and sexual orientation both impact survivors' experiences significantly and across all facets of the experience (Meyer, 2003).(Meyer, 2003). For starters, women are much more likely to be victims of sexual assault, and men are far more likely to be the perpetrators of sexual assault than women (Balsam et al., 2005; Johnson et al., 2016; Tjaden & Thoennes, 2000).(Balsam et al., 2005; Johnson et al., 2016; Tjaden & Thoennes, 2000). Given these relative rates in relation to sexual orientation, then, it might be expected that women who identify as bisexual or lesbian would thus experience sexual assault at lower rates than heterosexual women because these women have less exposure to men and male partners. However, this is not the case. The prevalence of sexual assault does vary by sexual orientation, but in fact bisexual women report experiencing sexual assault at higher rates than both lesbian and heterosexual women (Canan et al., 2021; Ford & Soto-Marquez, 2016; Martin et al., 2011). Overall, research indicates that 24-48% of bisexual women, 13-25% of heterosexual women, and 11-18% of lesbians report experiencing sexual assault during college (Ford & Soto-Marquez, 2016; Martin et al., 2011). More recently, studies have suggested that 63% of bisexual women, 49%

of lesbian women, and 35% of heterosexual women reported experiencing rape in their lifetime (Canan et al., 2021).(Canan et al., 2021). Alarmingly, when compared with other demographic characteristics, sexual orientation has been shown to be the most strongly correlated with sexual victimization (Canan et al., 2021).(Canan et al., 2021).

Sexual orientation also plays a role in the individual outcomes of sexual assault. Sexual assault leads to a variety of negative physical and mental health effects, and victims that hold a marginalized sexual orientation are more likely to experience these negative effects (Coulter et al., 2017; Cramer et al., 2012; Long et al., 2007). After trauma and/or violent crimes, LGBTQIA+ victims are more likely to report negative mental health outcomes (Cramer et al., 2012), and this trend includes sexual assault survivors, particularly bisexual women. Following a sexual assault experience, bisexual women had more depressive symptoms and higher levels of PTSD in comparison with heterosexual and lesbian survivors (Long et al., 2007). All of these statistics support the idea that sexual minority women need even more support when coping with a sexual assault experience.

Disclosure of sexual assault also varies by sexual orientation. Despite the particular importance of supporting queer survivors, research shows mixed findings regarding disclosure rates in heterosexual and lesbian, gay, and bisexual survivors. One common finding indicated that most sexual assault survivors who identify as a sexual minority disclosed their experience informally rather than formally (Grocott et al., 2022). Informal disclosure involves telling a family member, friend, or partner about the sexual assault, while formal disclosure refers to telling the police, medical professionals, or mental health counselors (Starzynski et al., 2005). Findings indicate that 64-94% of sexual assault survivors informally disclose to family or friends (Ahrens et al., 2009; Krebs et al., 2016), but less than 7% of survivors formally disclose (Orchowski & Gidycz, 2012). A review article reported that 9-44% of sexual and gender minority participants disclosed to an informal source, while a smaller percentage, 4-21%, disclosed to a formal source (Edwards et al., 2022). Informal disclosure is also more common than formal disclosure in heterosexual survivors (Starzynski et al., 2005). Understanding the most common type of disclosure in survivors is imperative to consider when conceptualizing resources and changes that could be made to improve the comfortability of survivors and encourage help-seeking. While some studies have indicated that lesbians were less likely to seek help than heterosexual women following a sexual assault experience (Bernhard, 2000), alternative studies have shown no significant difference in formal or informal disclosure between heterosexual and LGBTQIA+ survivors (Richardson et al., 2015). These mixed reports suggest that additional research is needed for clarity. This will allow for the improvement of resources available to sexual minority survivors.

It is further important to consider the outcomes of disclosure in sexual minority women. For example, even if disclosure does occur, bisexual women perceive less social support and are more likely to receive negative reactions when disclosing a sexual assault experience (Sigurvinsdottir & Ullman, 2016). In addition, victim blaming, or placing the responsibility of a crime on the individual affected, is more prevalent when the victim is gay or bisexual in comparison to a heterosexual victim (Davies et al., 2011; Morrison & Pedersen, 2020). This increase in blame in addition to reduced social support may further inhibit disclosure for queer women, as well as contribute to the increased negative mental health outcomes experienced.

It is clear that sexual orientation plays a role in sexual assault experience, disclosure, and support, but it is important to consider why this is the case. This may be explained by an inequity in both protective and risk factors between heterosexual and non-heterosexual individuals. Protective factors, such as a strong social support network, decrease one's risk of victimization, while risk factors increase this likelihood. LGBTQIA+ individuals have fewer protective factors and more risk factors than those who do identify as heterosexual (Blake et al., 2001; Mintz et al., 2021; Oshri et al., 2014). The inequity in protective and risk factors can be explained by Minority Stress Theory, an explanation of the heightened levels of stress experienced by those who are a part of marginalized groups due to pervasive discrimination and prejudice (Meyer, 1995). This theory suggests that people in minority groups experience more stress and are therefore more likely to struggle with their mental health and engage in unhealthy coping mechanisms such as using substances or engaging in risky sexual behavior. This ongoing stress of holding a minority identity increases the likelihood of individuals engaging in behaviors that increase the risk of sexual assault due to constant heightened stress as a result of discrimination (Blake et al., 2001; Oshri et al., 2014).

Minority stress for LGBTQIA+ individuals in particular may manifest as internalized homophobia. This concept can explain the increase in stress experienced by sexual minorities in particular, as well as the heightened risk of sexual assault faced by sexual minority women. Internalized homophobia can be

described as stigmatized self-perception LGBTQIA+ individuals have of themselves due to negative societal and cultural messages received about their identity (Meyer, 2003). These negative messages cause additional stress and may lead to additional risk behaviors in LGBTQIA+ individuals. In fact, similarly to minority stress, increased rates of internalized homophobia are related to a higher risk of having an unwanted sexual experience (Murchison et al., 2017). This increased risk of sexual victimization could be explained by the increase in risk behaviors such as a greater number of sexual partners and regular use of alcohol that are also correlated with higher levels of internalized homophobia (Baiocco et al., 2010; Combs-Lane & Smith, 2002; Martin et al., 2011; Rosario et al., 2006). When considering sexual assault among sexual minority women, it is important to understand the intersectionality of that identity. Intersectionality is a concept that emphasizes the interactions between various social identities (including race/ethnicity, gender, class, age, sexuality, religion, etc.) all affected by societal power structures. Therefore, those who hold multiple marginalized identities experience unique barriers. For the purpose of this study examining sexual minority women, as both a woman and a sexual orientation other than straight, these survivors face layered challenges (Hankivsky, 2022).

Given that women who identify as a part of LGBTQIA+ communities have higher risk for problems after sexual assault in addition to more issues surrounding support and disclosure, it is particularly important to understand their help-seeking process. To understand why people may choose to seek help or not seek help following a sexual assault, a model addressing health behaviors can be a useful tool. The Health Belief Model outlines various factors that may alter an individual's health behaviors (Green et al., 2020). In this model, the likelihood of taking action is affected by one's motivation, perceived susceptibility and severity of their condition, perceived benefits of taking action, and perceived barriers they may face when taking action. These factors are affected by the demographic variables and psychological characteristics of an individual, providing a comprehensive understanding of the variety of elements that may affect one's decision to take action (Green et al., 2020). In the case of survivors of sexual assault, action refers to seeking help through formal and informal disclosure.

It is integral to understand barriers survivors may face when disclosing their experience, especially for at-risk groups such as sexual minority women. All individuals who experience sexual assault may face barriers when disclosing their experience, regardless of sexual orientation. Barriers may include fear of experiencing stigma, feelings of shame or self-blame, belief that their experience was not serious, and desire to deal with their experience in their own way (DeLoveh & Cattaneo, 2017; Sabina & Ho, 2014). Feelings of self-blame and shame could be considered a perceived barrier by the Health Belief Model and decrease the likelihood that an individual seeks help. The barriers previously mentioned may be magnified for sexual minority women. This is because female sexual minority survivors face more victim blaming in comparison to heterosexual survivors, which may lead to additional self-blame and feelings of shame (Davies et al., 2011; Morrison & Pedersen, 2020).

In addition to barriers, perceived severity is a second factor from the Health Belief Model that can relate to help-seeking for sexual assault. For example, it is typical for survivors to believe their experience was not "serious enough" to report, lessening their likelihood of seeking help (Holland & Cortina, 2017). This phenomenon is also certainly affected by demographic variables such as sexual orientation. If an individual experiences greater levels of victim blaming and societal judgment surrounding their experience, they may feel that the severity of their experience is minimal. Sexual minority women are more likely to experience higher levels of victim blaming, and therefore the perceived severity of their experience may decrease. This could lead to less action, or help seeking, after a sexual assault experience for this group in particular.

A third factor from the Health Belief Model that is relevant in the context of disclosure is perceived benefits (Green et al., 2020). Perceived benefit refers to the idea that taking an action will in some way benefit the individual, in this case, for example, getting support from a loved one or getting resources by disclosing to a formal source. In survivors of sexual assault, sometimes perceived benefits can be low due to fear of experiencing stigma and a desire to manage their experiences independently (Holland & Cortina, 2017), which can limit help-seeking behavior. Perceived benefits of disclosing an experience also vary based on a survivor's sexual orientation and the resources available to a survivor. LGBTQIA+ survivors are more likely to disclose to others who are affirming of their identity rather than a non-affirming individual or organization (Hackman et al., 2020), however trusted organizations or individuals may not be available to all survivors. The higher rates of victim blaming among LGBTQIA+ survivors may contribute to the lack of specific affirming resources for this group. If no affirming support is available, a sexual

minority survivor may perceive less benefit in disclosure, and therefore not seek help.

Taken together, past research has shown that sexual assault has negative effects on those who experience it, and that there are unique risk factors and barriers for sexual minority women that affect their experience and subsequent outcomes. These differences may be accounted for by the effects of minority stress and internalized homophobia faced by sexual minority women, as well as negative societal stereotypes that lead to heightened levels of victim blaming of bisexual women. As explained by the Health Belief Model, high perceived barriers and low perceived severity and benefit may decrease one's likelihood of taking action. Consequently, further research is needed to better understand and mitigate the connection between negative societal perceptions of LGBTQIA+ women and the decreased likelihood that these women seek help in comparison to those who do not identify within these communities.

To increase our understanding of this phenomenon, this study aims to answer the question: How does an individual's identity affect their perception and experience with unwanted sexual attention? More specifically, this study will explore perceived benefits and barriers to disclosing a sexual assault experience, recommendations received by queer survivors and provided by the general public, and experience with unwanted sexual attention in those who hold a queer identity. Understanding these variations will allow us to gain a better understanding of how to create interventions and education programs to specifically target helping this group that is at a greater risk of both sexual assault and negative outcomes. The study was qualitative and exploratory in nature to give voice to individuals in this high-risk population.

Methods

Participants

Participants included nine college students from a small private university who self-identified within the LGBTQIA+ community. A majority of participants identified as Caucasian/white Non-Hispanic (89%), with remaining participants identifying as Asian Non-Hispanic (11%). The class rank of participants represented all four years of undergrad, with 44% seniors, 22% juniors, 11% sophomores and 22% freshman. In Table 1, participants are listed via their code name, gender identity, sexual orientation, and survivor status.

Table 1
Participant Information

Name	Gender Identity	Sexual Orientation	Survivor Status
Parker	Cisgender Woman	Gay/Queer	Survivor
Quinn	Woman	Lesbian/Queer	Survivor
Logan	Woman	Lesbian	Survivor
Alex	Genderfluid	Queer	Survivor
Cameron	Intersex/Woman	Bisexual	Survivor
Riley	Nonbinary	Bisexual/Queer	Survivor
Avery	Woman/Genderqueer	Lesbian	Nonsurvivor
Carson	Gender-ambiguous	Gay/Queer	Nonsurvivor
Blake	Cisgender Woman	Bisexual	Nonsurvivor

Procedure

Elon University's Institutional Review Board approved this study prior to participant recruitment. Convenience and snowball sampling was used to recruit participants. All participants attended Elon University and were contacted through the distribution of study materials through LGBTQIA+ students and organizations. Participants had to self-identify as LGBTQIA+ and as female, nonbinary, or any other gender identity that was not male to participate in this study. We distributed advertisements across the university's campus via physical and virtual flyers to LGBTQIA+ groups and organizations and to the general student population. Flyers included a link to a preliminary survey. This survey ensured that interested participants met the inclusion criteria, provided researchers with informed consent, and allowed researchers to separate participants into focus groups. This survey also asked participants to report demographics and assault history. Interested individuals who met the criteria were contacted via email and assigned to a focus group based on their availability and survivor status. Focus groups were conducted via Zoom,

and participants were informed before joining the meeting and at the beginning of the focus group that they were welcome to change their Zoom name and turn their camera off to preserve confidentiality if preferred. Individuals who participated in a focus group received a $10 gift card.

Measures

We used the Sexual Experiences Survey (Koss & Oros, 1982)(Koss & Oros, 1982) to separate interested participants into focus groups composed of either survivors or individuals who had not experienced sexual assault. In this study individuals who indicated experiencing anything beyond unwanted touching were classified as survivors. We altered the survey language in to be more inclusive, and the Sexual Experiences Survey has been validated to assess sexual assault and rape in lesbian and bisexual women (Canan et al., 2020). Based on participant and researcher availability, three focus groups with three participants each were formed. Of these groups, two were composed of survivors while one group contained nonsurvivors. Each group was asked the same questions, as shown in Table 2, with framing appropriate for survivor/nonsurvivor status. Survivors were also asked to discuss what recommendations they had actually received.

Qualitative Analysis

Audio recordings from each Zoom focus group meeting were transcribed via Otter.ai software. We coded the transcripts following inductive thematic analysis (Braun & Clarke, 2006). This method was utilized to analyze the qualitative data from the three focus groups, and included six steps, starting with step one, researchers familiarizing themselves with the data. This process began with each researcher reading through the transcripts and taking individual notes, identifying possible themes within each question participants were asked. This process allowed researchers to complete step two, the generation of initial codes, and step three, the search for themes. Step four involved reviewing and refining themes, made possible through conversation between researchers that allowed themes to be solidified and further understood in relation to other themes and subthemes that emerged. Through this process, researchers last completed step six, defining and naming themes through discussion. Unless specifically noted, each theme was brought up across focus groups.

Results

Barriers to Disclosure

Participants were first asked about general barriers to disclosure of sexual assault. When asked about barriers to sexual assault disclosure, participants' answers could be categorized into the theme *Concern Around Social Reactions*. This was expressed by participants as fear of judgment of themselves or their situation. More specifically, when expressing fear of being judged themselves, participants expressed feelings

Table 2
Questions For Participants

Questions for Survivors	Questions for Nonsurvivors
What are the barriers to disclosing sexual assault or unwanted sexual experiences?	What are the barriers to disclosing sexual assault or unwanted sexual experiences?
What are perceived benefits to disclosing a sexual assault or unwanted sexual experience?	What are perceived benefits to disclosing a sexual assault or unwanted sexual experience?
What kinds of recommendations did you receive upon disclosure?	
What recommendations would you provide to others who have this kind of experience?	What kind of recommendations would you provide to a friend who experienced a sexual assault or unwanted sexual experience?
Are there ways in which your sexual orientation or gender identity affected your decision to disclose or to not disclose?	Are there ways in which one's sexual orientation or gender identity might affect their decision to disclose or not disclose?
Are there ways in which your sexual orientation or gender identity affected your experience of unwanted sexual behavior?	Are there ways in which sexual orientation or gender identity might affect someone's experience of an assault?

of embarrassment, experiencing stigma, and fear of being blamed or not believed. Some participants expressed a fear of their situation being judged, reporting concern that disclosure would result in pressure not to report, a misunderstanding of LGBTQIA+ relationships and identities, or a worry that others would feel that too much time had passed since the experience. Additionally, the nonsurvivors identified a lack of benefits as an additional barrier to disclosure.

Benefits to Disclosure

Participants were also asked to relay their perceived benefits of disclosure. The two themes that emerged from these responses were *Support* and *Justice*. Participants expressed the theme *Support* often throughout focus group discussion. This theme emerged from reports that disclosure could aid in the processing of emotions and gaining help and assistance through professional services. In addition, participants discussed that disclosure could provide them with emotional and social protection. Avery explained this, saying "...the more people who know about something, the more people who can ...help protect you or ...keep you safe in any situation ...not even like in a physical sense, necessarily, but just in the sense of ...it's nice to have somebody where ...if you're triggered by something, or if something's going on that you can ...turn to and they know what's going on enough to be able to help you through it." Finally, participants noted that disclosure could assist in finding those with similar experiences, aiding survivors in feeling less alone. The theme *Justice* was expressed by participants as they talked about disclosure leading to holding perpetrators accountable. Parker described this saying "...let it hold the person accountable for their actions."

Recommendations Received by Survivors

Participants in the two focus groups composed of survivors were asked what recommendations they received after disclosing their experience. Themes included *Formal Support, Informal Support,* and *Lack of Support*. *Formal Support* developed from the common recommendation received by participants to seek counseling and therapy. *Informal Support* included recommendations to seek support from family and friends. This theme also included one participant's recommendation to seek informal justice through disclosure to family, friends, or colleagues of a perpetrator to achieve justice. The theme *Lack of Support* emerged as participants reported experiencing invalidation of their experience and the discouragement to further disclose. Participants indicated that following disclosure, the validity or nonconsensual aspect of their experience was often questioned. Cameron expressed this, saying "...I was kind of just told to shut up personally." Additionally, participants reported individuals being unsure of how to react following disclosure. Parker shared "...my parents didn't know how to approach situations. And then they just kind of were like, extra delicate around me in a way that was unnecessary." Quinn also reported this, saying "...they just kind of were like, oh, you know, if you need anything, let me know. But also ...telling me that ...oh, this doesn't usually happen like this, this, you know, they tried to tell me ...Oh, this is ...not normal, which didn't really help in the moment when it was ...something that had just happened. And so obviously it does still."

Recommendations Provided to Others

All participants were asked which recommendations they would provide to someone who experienced an unwanted sexual experience. Themes in this domain included *Formal Support*, *Informal Support*, and *Tentative Justice*. Like recommendations received, most participants discussed recommending *Formal Support* in the form of therapy or professional counseling. Also similarly to recommendations received, participants recommended seeking *Informal Support* from friends and family. *Tentative Justice* arose as a theme from survivors' recommendations to gather evidence and consider a formal report if that is what they feel is best. This theme name was chosen by the researchers in response to the varying opinions and advice surrounding formal reporting given by participants. Cameron expressed "...I would recommend someone reports anyway, just because ...at the time, you know, I didn't really like understand ...why I should report, but it's something I regret a lot. So I'd say report even if you don't ...feel like it in the moment. But if you don't have a lot of I don't want to say evidence because ...your word is evidence. But if you don't have ...a lot of ...hard evidence, it might not be worth ...rehashing everything." Alex expanded on this saying "I would also have some hesitation with suggesting that anyone report formally, however, I would if that person was really uncomfortable with the idea of reporting it formally, I think my first thought would be to gather the evidence anyway, have screenshots or whatever, you can write down whatever you can, so that maybe if you don't want to report it, now, you can report it later."

Sexual Orientation and Gender Identity: Effect on Disclosure

Participants were also asked if their sexual orientation or gender identity did or would affect their decision to disclose. Themes that emerged included participants who answered no because they were *Not Out,* no because their experience was *Not Identified as*

Nonconsensual, yes due to *Societal Issues,* or yes due to *Queer Community*. Some participants said no as they were not out to themselves at the time, resulting in the theme *Not Out*. The theme *Not Identified as Nonconsensual* refers to participants who did not disclose due to their lack of recognition that their experience was not consensual. The theme *Societal Issues* emerged from participants' expression that societal pressures on women discouraged them from disclosing. This theme was named by researchers due to the common report that societal pressures discouraged disclosure, particularly for women or those perceived as feminine. Survivors expressed that their identity as a queer person did not affect their decision to disclose, but their identity as a woman did. Quinn stated "...it doesn't relate to ... a queer identity ...but yeah, definitely recommendations typically were aimed towards women." The focus group of nonsurvivors uniquely reported that considerations of someone's identity should be considered when providing them with support following disclosure.

The theme *Queer Community* was formed in response to participants who expressed that their identity did affect their disclosure decision. In particular, participants reported that they were more comfortable disclosing to other queer individuals due to the increased empathy and increased likelihood of a common experience. As stated by Alex,"...I really only disclose anything to my friends or partners, who are almost all queer in one way or another. And I think that shared understanding made me more willing to disclose to them whereas to a straight or cis person, there's just a level that they wouldn't understand." Cameron expanded on this, saying "I feel like if you're queer, you, you've either experienced it, or you at least know someone personally, who's experienced gender violence."

Sexual Orientation and Gender Identity: Effect on Experience

When asked if their sexual orientation or gender identity did or would affect their experience of unwanted sexual behavior, participant responses were reflected in one of three themes: No because they were *Not Out*, yes due to fear of *Targeting* of a particular identity, or no but their experience led to an *Adjustment of Gender/Sexuality Presentation*.

Like above, the theme *Not Out* was drawn from participants who reported their sexual orientation and/or gender identity did not affect their experience due to them not being out at the time of the assault. The theme *Targeting* reflected participant reports of fear of targeting due to gender identity, in particular the increased targeting of trans women and the added discomfort of unwanted attention from men as a lesbian due to their lack of attraction. Carson stated "...women are disproportionately affected. And there are other cases of assault, like trans women are also targeted a lot of times and so like, definitely, people experience things differently, and cisgender men are disproportionately affected less. Just because that's how society has been set up." While Avery said "... it just feels ...strange to me, because ...I'm a lesbian. ...it's just weird to me when ... that kind of attention comes from a man. And ...not that it would be better from a woman or anything like that. But it's just like, different. And it's a different processing that happens in my brain."

The theme *Adjustment of Gender/Sexuality Presentation* reflected participant reports of changes in their sexuality and or gender presentation following their experience of sexual assault. In part, this theme captured participant reports of changing their sexuality and/or gender presentation to discourage male attention or to seek male validation following their experience. Participants varied in type of adjustment, as some expressed they presented as more masculine, while others reported presenting more feminine. For example, Quinn stated "after the first like main experience, I really ...started to embrace more ...masculine parts of my gender identity. Although I still identified as ...a cisgender woman throughout the whole process, I ...expressed a lot less femininity and more masculinity at times. And ...I tried to do that. And a lot of times, it was as a way of like, I wanted men to leave me alone." However, on the other hand, Alex stated "I think that I ended up not accepting being gender fluid for a while because I had this underlying trauma where I wanted to be feminine and pretty and desirable."

This theme also takes into account participants' explanation that their experience caused them to later feel less comfortable with men and safer with women. Cameron, who identifies as bisexual, stated "...for a really long time, I was ...unable to ...handle the concept of a man touching me. So I identified as a lesbian for a very long time because I couldn't tell the difference between ...the trauma ...repulsed touch, versus like attraction. So eventually, I started ...somewhere in my healing process, I realized my attraction to men." Logan, who identifies as a lesbian, also mentioned "...it just kind of made me more uncomfy towards the opposite gender, which so I don't think it was ...my gender and sexuality influenced that. I think it was almost the other way around. But I think it made me ...feel safer with the same gender and probably pushed me closer to ...coming out to myself." Participants agreed that their experience affected their identity in certain ways, as Cameron expressed, "I wouldn't

necessarily say my identity influenced it, I would say what happened influenced my identity…" and Alex reiterated, saying "…it shaped my experience as a person more so than as a queer person."

Discussion

Studies have shown that queer women experience sexual assault at higher rates, face more victim blaming, and report more negative mental health outcomes following an assault than straight women (Canan et al., 2021; Ford & Soto-Marquez, 2016; Long et al., 2007; Martin et al., 2011). We conducted the current study to gain a deeper understanding of the variation in sexual assault experience and disclosure among queer individuals and investigate ways to better support queer survivors given that they receive less support and more blame. Participants were asked to describe perceived barriers to disclosing a sexual assault experience. It was found that participants reported barriers such as fear of being judged, stigma, being blamed or not believed, and feelings of embarrassment. These barriers are consistent with the current literature and therefore these results were expected (DeLoveh & Cattaneo, 2017; Sabina & Ho, 2014). The current literature provides knowledge of general barriers faced by survivors, while this study confirms that queer individuals perceive the same barriers, through the theme *Concern Around Social Reactions*. Additionally, this data provides a deeper understanding of the perception of these barriers by queer participants through qualitative data that highlights the voices of these individuals. It is important to know the barriers to disclosure that are perceived by survivors and queer survivors in particular because this can guide the creation of resources or interventions that minimize said barriers, encouraging help seeking among survivors. Participants also reported that a misunderstanding of queer relationships may contribute to barriers faced by queer survivors. This further highlights the need for more specific research to examine this phenomenon.

In addition to asking questions to understand participants' perceived barriers to disclosure, participants were also asked to describe benefits to disclosure. Participants reported benefits to disclosure such as gaining support through family and friends or professionals and getting justice through holding a perpetrator responsible. Holding a queer identity was not mentioned as a factor by participants. These responses were also in line with previous research and are important to note as a comprehensive understanding of the benefits perceived by survivors can also guide resource creation, implementation, and advertisement. In general, the knowledge of survivors' expectations following help seeking can help programs, resources, and individuals meet and exceed those expectations, improving survivors' support following a sexual assault experience.

Additional questions aimed to gain a deeper understanding of how participants' gender identity and sexual orientation affected their disclosure decision and experience. This was assessed to gain understanding of how minority stress theory might play a role in these experiences (Meyer, 2003, p. 2). We expected that due to minority stress theory, holding a queer identity would affect the sexual assault experience and disclosure decisions of participants, potentially leading to lower rates of disclosure and an experience that differed from their straight counterparts (Meyer, 1995, p. 199). However, this is not what participants indicated in their responses. Queer participants instead expressed that their experience affected their identity more than the other way around. Participants explained a variety of responses following their experience that could be categorized into the theme *Adjustment of Gender/Sexuality Presentation*. The adjustments described varied by participant, as some expressed a change in the label of their sexuality due to a discomfort around men while others reported a difference in gender expression, either favoring a more feminine or more masculine presentation, as an attempt to repel men or a desire to fill societal expectations. Participants' perception of their own identity, and in turn their presentation of said identity, was altered following their experience. These unexpected results counter the prediction of how minority stress may influence a survivor's experience, providing an alternative result in need of further investigation. This valuable qualitative data contributes insight into the experiences and processing of queer survivors. This will allow for the creation of more helpful resources for said survivors and shape future research projects aimed to gain further understanding of this phenomenon.

Participants were also asked to indicate factors that impacted their disclosure decision, and our expectation was that minority stress would relate to decreased disclosure in queer survivors (Meyer, 1995). However we were once again surprised by the results. Participants reported that their disclosure decision was more prominently impacted by their gender identity as a woman, or societal perception as a woman, rather than their sexuality. In fact, many participants reported that their sexuality did not impact their disclosure decision. Their gender identity, however, did have an impact due to societal pressures on women that increase blame and discourage disclosure surrounding sexual assault. This

highlights the major effect of patriarchal ideals and societal concepts of femininity on survivors and their disclosure decision. This finding suggests that interventions should be aimed at altering these societal pressures on women to create the most positive change in survivors' comfortability with disclosure and help seeking.

A few of the themes that emerged require additional research to further understand its relationship with a queer identity, particularly themes *Not Out* and *Not Identified as Nonconsensual*. The theme *Not Out* brings to attention the possibility that survivors who were out at the time of their unwanted sexual experience have a very different experience, and a different relationship with their queer identity, than survivors who were out. The theme *Not Identified as Nonconsensual* emerged as participants reported that they did not identify the experience as nonconsensual which is true of many survivors but also may have been affected by participants' evolving gender identities and sexualities.

Participants who reported that their sexuality did have an effect on their disclosure decision explained that they were more likely to disclose to other queer individuals. This provides a possible explanation for lower rates of disclosure among queer survivors, as a lack of access to trusted queer individuals or queer resources may decrease the likelihood of disclosure. This finding highlights the importance of queer and queer affirming resources for survivors. In addition, this finding allows us to consider the interaction between survivors' identities. Queer women face societal pressures discouraging disclosure due to their gender identity, and they also may only feel comfortable disclosing their experience to another queer person, further decreasing the likelihood of seeking help. The combination of these barriers for queer women contributes to their heightened risk of negative mental health outcomes and minimized likelihood of disclosure. This further emphasizes the need for focused interventions put in place for this at risk group.

Clinical Implications

Several of the themes that emerged provide suggested directions to take services for survivors. The theme *Queer Community* highlighted the increased comfortability of queer survivors to disclose their experiences to those within the LGBTQIA+ communities. This finding emphasizes the importance of the existence of queer healthcare providers, as well as queer affirming healthcare services. The availability and advertisement of these services, from mental health to physical health, is crucial for many queer survivors to feel comfortable seeking help. More education surrounding gender and sexuality, and the challenging of societal norms is necessary to improve care for survivors of all gender identities and sexual orientations. The theme *Societal Issues* also brings to light the need to challenge societal blame pushed on women when it comes to sexual violence, as many participants voiced that due to being socialized as a woman, they felt discouraged to disclose and seek help.

Limitations and Future Directions

It is possible that the generalizability of the results of this study are limited due to the nature of the sample obtained. Although the sample size was intentionally limited given the qualitative and personal nature of data collection, it is possible that the responses obtained do not reflect the full spectrum of experiences across gender and sexuality. The study focused on female-identified and gender minority experiences, and there is room for expansion to be even further inclusive of multiple and intersecting identities. Additionally, all participants were students at one private university in the Southeast. While useful information was obtained, it is possible that culture, beliefs, and ideas vary in individuals in other settings. Finally, only individuals who had disclosed and felt comfortable discussing their experience participated, potentially restricting the variation in experiences of focus group participants. Further research is essential to further explore the experience of sexual assault in the larger queer community, and particularly needed to investigate the theme of participants' adjustment of gender/sexuality presentation in response to a sexual assault experience. The emergence of this theme provides a complex glimpse into the experience of queer survivors and their viewpoint on their identity and experience, but further examination would be beneficial to deepen our understanding of this phenomenon.

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